

CLINICAL FINDINGS

Skin and Mucous Membrane Involvement

The characteristic skin lesion of bullous pemphigoid is a large, tense blister that arises on normal-appearing skin or on an erythematous base. These lesions most commonly occur on the lower abdomen, inner or anterior thighs, and flexor surfaces of the forearms, although they can appear anywhere on the body. The bullae are typically filled with clear fluid but may be hemorrhagic. After rupture, the erosions usually reepithelialize well and do not expand peripherally, unlike in pemphigus vulgaris. New vesicles may, however, develop at the edges of resolving lesions. Bullous pemphigoid lesions do not scar, and pruritus is commonly present, though not universally.

In some cases, the erythematous component predominates, and patients may initially present with urticarial plaques, particularly early in the disease course. The erythema can have a serpiginous pattern with peripheral blistering and may be extensive. Healing typically begins in the center of the lesions and may be accompanied by hyperpigmentation.

Mucous membrane involvement occurs in approximately 10% to 35% of patients and is almost always confined to the oral mucosa, particularly the buccal mucosa. Lesions may appear as intact blisters or, more commonly, as erosions. These do not scar and are usually not widespread. Notably, the vermilion border of the lips is rarely affected, distinguishing it from erythema multiforme.

Unusual Clinical Presentations

Bullous pemphigoid can present in atypical forms, all confirmed by characteristic immunofluorescence findings. In some cases, the disease is localized, most frequently to the lower legs. Patients with localized disease have antibodies against the same pemphigoid antigens (BP180 and BP230) as those

with generalized disease and may either remain localized for years or progress to generalized involvement.

A distinct variant occurs in young girls, characterized by blisters and erosions limited to the vulvar and perivulvar regions. As in other localized and childhood forms, autoantibodies target the classic pemphigoid antigens.

Early in the disease, some patients may exhibit only urticarial or eczematous lesions without frank bullae. Other rare presentations include erythroderma, prurigo nodularis-like lesions, vegetating lesions, and dyshidrotic dermatitis-like eruptions. Despite the atypical morphology, the immunopathological findings—such as linear deposition of IgG and C3 along the basement membrane zone and reactivity with BPAG2 (BP180)—remain consistent with bullous pemphigoid.